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US savings from international reference pricing for pharmaceuticals crucial to women's health

Prescription drug prices in the USA are among the highest in the world. This burden falls disproportionately

on women, who incur higher lifetime health-care spending than men while facing persistent income and wealth disparities.¹ Women uniquely bear the financial weight of high-cost, sex-specific conditions, particularly breast and gynaecological cancers, as well as recurring expenses for contraception, menopause therapies, and gestational diabetes management. Breast cancer, for example, occurs in one of every eight US women during their lifetime² and is the most expensive type of cancer to treat in the USA.³ International reference pricing, which benchmarks US drug prices against other high-income countries, has gained bipartisan interest as a potential

solution.⁴ In the context of women's health, such a pricing policy could provide meaningful financial relief and improve access to care for women.

Building on our previous analysis estimating US savings on outpatient prescription pharmaceuticals from international reference pricing,⁴ we evaluated savings specifically for major women's diseases and reproductive care. We modelled a policy scenario consistent with the Prescription Drug Price Relief Act of 2025, capping US drug prices at the lower of the current US net price or the median list price across Canada, France, Germany, Japan, and the UK. We analysed all branded products indicated for

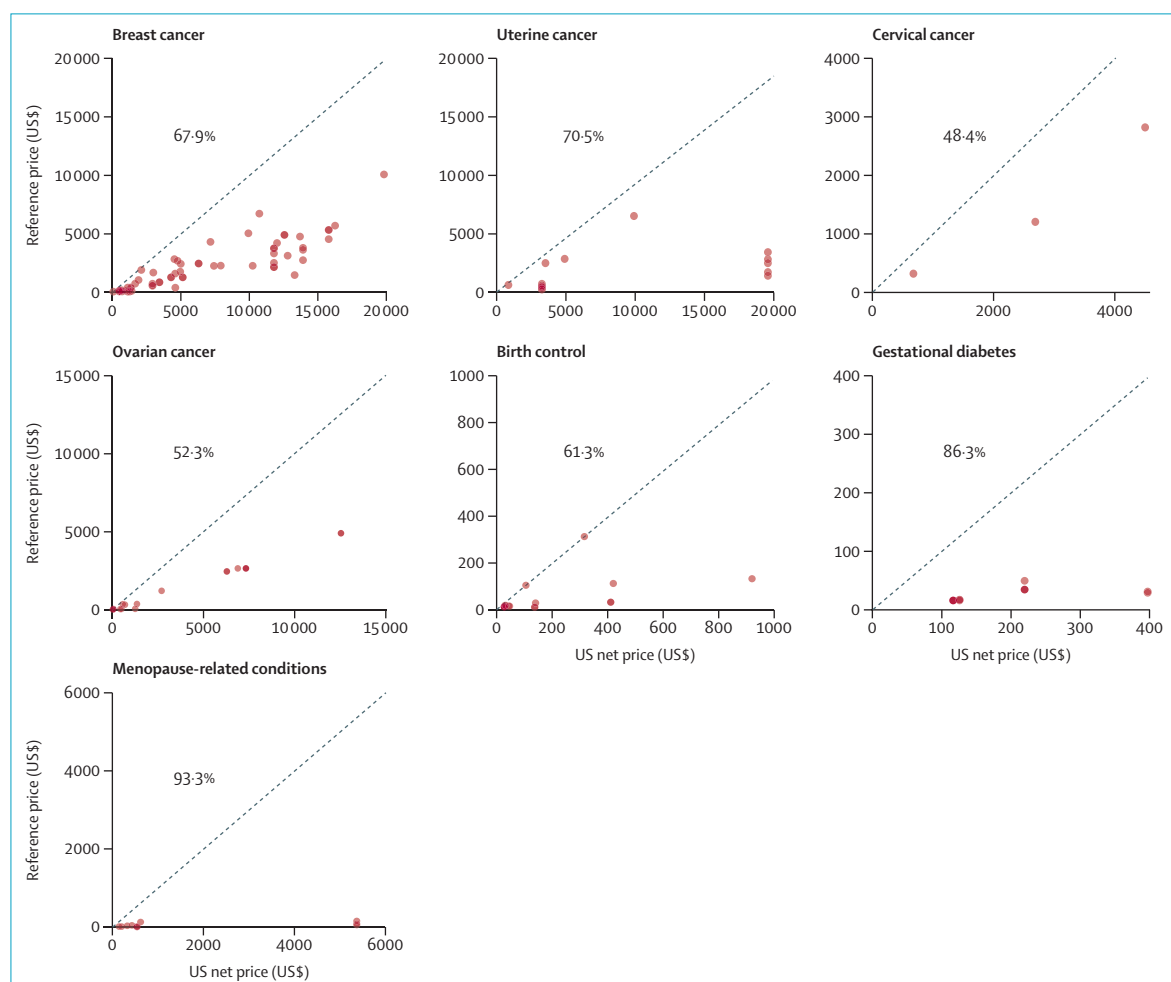


Figure: Price comparison for branded drugs with international equivalents

Each point represents a National Drug Code; percentages indicate the average cost reduction across all drugs with international reference pricing. The dashed line indicates a reference price equal to the US net price. Breast cancer includes adjuvant, metastatic, bone metastases, and palliative settings; birth control includes routine and emergency contraception; menopause-related conditions include hot flashes and other perimenopausal and postmenopausal symptoms.

breast cancer, uterine cancer, cervical cancer, ovarian cancer, contraception, gestational diabetes, and menopause-related conditions. US net prices for these drugs were estimated by combining their list prices in 2022 with the published average net-to-list price ratio of 84.3% for branded drugs in the USA.⁵ Across all branded drugs with international equivalents, international reference pricing would reduce disease-specific net prices dramatically, with reductions ranging from 48.4% for cervical cancer to 93.3% for menopause-related conditions (figure).

For breast cancer and birth control, two major areas with substantial drug spending, we projected annual savings using nationally representative utilisation data from the 2022 Medical Expenditure Panel Survey.⁴ For breast cancer, we included female respondents who reported ever being diagnosed with breast cancer and were prescribed antineoplastics. For birth control, we included female respondents who reported receiving contraceptive counselling in the past 12 months and were prescribed drugs with a contraceptive indication in the sex hormone or gynaecology therapeutic areas. Comparing current US net expenditures with an alternative scenario in which each given US drug is used at its current level but priced at the reference price, we estimated annual savings of US\$1.2 billion for breast cancer and \$111.1 million for birth control.

These findings suggest that aligning US prescription drug prices with those in other high-income countries could yield substantial savings for women and the health-care system. Lowering drug costs could improve women's health by expanding access to treatment, supporting continuity of care, and reducing health disparities. As policy makers consider strategies to address persistently high drug prices in the USA, our results underscore the potential value of international reference pricing as a component of broader drug pricing reforms.

We declare no competing interests.

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A cultural competency toolkit for menopause in the UK

A fundamental principle when supporting women who are experiencing menopause is to use a life-course approach combined with cultural competence.¹ Evidence is required to guide this support for all women, but empirical information relating to the menopause transition for women in minority ethnic groups is sparse.²

In 2024, we began a research project called Menopause and Ethnicity to establish an understanding of how women from diverse ethnic communities experience the menopause transition. The project sought to extend the focus beyond the biological context by acknowledging not only that culture and society are important in shaping embodied experiences but also that there is great diversity in the experiences of women.³

Panel: Six key points in the cultural competency menopause toolkit for health-care professionals

- 1) Myths. For example, that women from minority ethnic groups prefer traditional treatment.
- 2) Facts. For example, there is huge heterogeneity in experiences relating to menopause between minority ethnic groups and for individual women within minority ethnic groups.
- 3) Stigma, taboos, and beliefs. For example, regarding mental and sexual health in relation to menopause for minority ethnic groups.
- 4) Language. For example, terminology related to menopause can vary both in context and meaning between minority ethnic groups and for individual women within minority ethnic groups.
- 5) Starting a conversation and building rapport. For example, women from minority ethnic groups would particularly benefit from health-care professionals who initiate conversations related to menopause, especially in light of stigma and taboo.
- 6) Holistic conversation and care. For example, women from minority ethnic groups would particularly benefit from health-care professionals who include conversations about alternative remedies, which are widely used.

We interviewed and ran focus groups with 60 women from minority ethnic backgrounds (women with Black origins [n=30] and Chinese heritage [n=30]) who were residing in the northwest of England; we also interviewed four health-care providers (three National Health Service general practitioners and one private psychotherapist). Recurring themes—or attention points—were identified from these discussions (panel) and highlighted the importance of a life-course approach and cultural competence when supporting women from minority ethnic groups. From these themes, we developed a cultural competency toolkit for menopause (appendix).

The toolkit is available for all readers to use and disseminate across sectors and will help broaden the clinical outlook on menopause. We envisage that the toolkit will encourage an evaluation of ethnocentric views, provide skills to communicate, raise awareness of cultural diversity, and promote adaptability and flexibility so that care and support for women from minority ethnic backgrounds can be optimised, both in the menopause transition and later in the life course.

See Online for appendix